DBA RESEARCH PROPOSAL

Supervisory Emotional Intelligence and Emotional Exhaustion in the Veterans Health Administration

An inductive qualitative study of how emotionally intelligent supervisory behavior is enacted, interpreted, and linked to clinician depletion or renewal.

Donald Joseph Hupp Jr.
Supervisory EI & Emotional Exhaustion
Doctor of Business Administration Program
Weatherhead School of Management, Case Western Reserve University
Advisor: Richard E. Boyatzis, PhD

PRESENTATION ARC

Problem → Gap → Relational Mechanism → Method → Contribution

This talk reframes emotional exhaustion as a relationally mediated organizational process rather than solely an individual condition.

WHY THIS MATTERS

Mental health clinicians in the VHA work under sustained emotional demand.

They are often balancing trauma-focused care, crisis response, heavy documentation, productivity expectations, and the procedural demands of a large federal healthcare system.

STARTING POINT

Emotional exhaustion is not simply an individual stress reaction.

It emerges inside organizational life, where emotional labor and institutional pressure accumulate over time and shape both clinician well-being and continuity of care.

THE PROBLEM

Most burnout responses focus on the individual.

Resilience training

Build the clinician's personal capacity to cope with stress.

Mindfulness or self-care

Strengthen internal regulation and recovery practices.

Workload adjustments

Reduce demands where possible through staffing or redesign.

These efforts matter. But they often underemphasize the relational and supervisory context in which emotional exhaustion is actually experienced.

Burnout is not only individual. It is also relational.

- Supervisors mediate organizational expectations and shape how demands are interpreted.
- They influence emotional climate, role clarity, psychological safety, and performance pressure.
- Much of the literature remains variable-centered, asking whether leadership correlates with burnout rather than how these processes unfold in practice.

The unresolved question is not simply whether leadership matters, but how supervisory behavior becomes experienced as supportive, constraining, renewing, or depleting.

CORE ARGUMENT

SUPERVISORY BEHAVIOR

Emotionally intelligent actions in daily interaction

Empathy, coaching, self-regulation, attunement, supportive communication.

→

RELATIONAL EXPERIENCE

How clinicians interpret supervision

Support or pressure, clarity or ambiguity, trust or emotional constriction.

→

EMOTIONAL OUTCOME

Renewal or exhaustion over time

Repeated interactions may buffer emotional strain or intensify depletion.

RESEARCH QUESTION

How are supervisors' emotionally intelligent behaviors enacted and interpreted within supervisory relationships, and how do these interactions shape mental health clinicians' experiences of emotional exhaustion within the Veterans Health Administration?

The emphasis is explicitly processual: enacted, interpreted, and shaped. This moves beyond traits and beyond simple correlation.

THEORETICAL FRAMING

Behavioral Emotional Intelligence

Following Boyatzis, emotional intelligence is not treated only as an internal psychological capacity. It is also an observable set of competencies enacted in relationship.

Intentional Change Theory

Supervisory interactions may activate states associated with openness, learning, compassion, and renewal or, alternatively, stress, defensiveness, and emotional constriction.

Resonant Relationships

Shared vision, compassion, and relational energy may function as the mechanism through which emotionally intelligent supervision affects well-being.

Sensitizing Frameworks

Role theory, JD-R, and Conservation of Resources orient attention toward demands, ambiguity, and resource trajectories without functioning as rigid coding templates.

CONCEPTUAL MODEL

A relational pathway from supervision to exhaustion

ORGANIZATIONAL DEMANDS

Caseloads, documentation, metrics, bureaucracy

These pressures structure the context in which supervision occurs.

translated through

SUPERVISORY MEDIATION

Framing, clarifying, coaching, pressuring, supporting

Supervisors convert system demands into lived experience.

experienced as

CLINICIAN EXPERIENCE

Role clarity, strain, support, depletion, renewal

This is where emotional exhaustion becomes meaningful and cumulative.

The conceptual move here is simple but important: from leadership as a personal trait to supervision as an interactional process embedded in organizational life.

METHODOLOGY

Inductive qualitative design

1

Purposeful, role-stratified sampling

Clinicians, first-line supervisors, mid-level managers, and chiefs provide multiple vantage points on the same supervisory system.

2

Semi-structured interviews

Participants describe emotionally significant supervisory interactions, support, pressure, role clarity, and work strain in their own words.

3

Constant comparison

Analysis moves within and across role groups to identify convergent patterns, divergence, and linked process across hierarchy.

4

Analytic memoing

Memoing supports conceptual development and a process account of how supervisory interactions contribute to depletion or renewal over time.

WHY THIS DESIGN FITS

This study is designed to capture what surveys often flatten.

It preserves context, meaning, sequence, and interpretation. That is essential when the phenomenon of interest is not simply burnout, but the supervisory processes through which exhaustion becomes lived and understood.

ANALYTIC ADVANTAGE

- Shows how support becomes support—or fails to.
- Explains process, not merely association.
- Preserves the bureaucratic context of the VHA.
- Allows unexpected patterns to emerge rather than be forced.

EXPECTED CONTRIBUTIONS

Theoretical Contribution

Reframes emotional intelligence as relationally enacted within supervisory systems rather than merely possessed by individual leaders.

Empirical Contribution

Illuminates how emotional exhaustion develops through repeated interaction, sensemaking, and organizational mediation in a large bureaucratic healthcare system.

Practical Contribution

Supports leadership development aimed not only at performance but at the relational conditions that help clinicians remain emotionally sustainable.

CLOSING THOUGHT

Emotional exhaustion is not inevitable. It is shaped—interaction by interaction—inside supervisory relationships.

This study asks a practical and theoretical question at the same time: What kinds of supervisory relationships help clinicians remain human, effective, and emotionally sustainable in demanding systems?

Thank you.
Questions & Discussion